CUSTOMER CARE LEARNING · PROPOSAL

WN7 Rollout & Reinforcement Plan

How the WN7 recovery intervention moves from readiness to sustained execution, who is accountable at each step, and what evidence decides whether it moves forward.

Prepared for Dana Ortiz, Director of Customer Care

Proposed design · owner confirmations pending

EXECUTIVE OVERVIEW

Eligible customers aren't reliably reaching recovery

The WN7 recovery path exists, but most eligible General Care calls still end in generic troubleshooting: **115 of 204** eligible calls (56%) in the September 21–30 Orbit call sample. This plan introduces a practical intervention, tests it with a small group, and expands only as evidence and capacity allow.

> **Reviewed WN7 calls show more consistent recognition, eligibility verification, appropriate recovery decisions, and clearly documented follow-up than the pre-rollout baseline. Customer outcomes show promising improvement, while service capacity remains manageable.**

Delivery
The intervention reaches Guides. Attendance and completion show reach, not proficiency.

Guide execution
Guides recognize, verify, choose the right path, and complete or document the recovery.

Customer recovery
More eligible customers leave with a completed recovery or owned, explained follow-up.

Operational capacity
Queues, wait times, and Lead and Specialist load stay manageable.

What's being rolled out

The Guide Call Path is the central resource; everything else introduces, practices, or reinforces it.

Lead working meeting
25-minute, case-based alignment on what good looks like and how to coach it

Team huddle + slide
Leads introduce the WN7 sequence and open dialogue with Guides

Digital practice scenario
Individual, in scheduled work time; write the decision, compare to a model answer

Call Path on live calls
In-call decision support, with Guide → Lead live support

Coaching + call review
Leads observe execution, diagnose gaps, coach or escalate

Five phases, each closed by a decision gate

1 **Prepare** — Readiness conditions confirmed by their owners

2 **Pilot** — 2–3 Lead teams; full intervention, live calls

3 **Observe & Adjust** — Quality-led evaluation against the baseline

4 **Expand** — Successive Lead-team waves, dual approval

5 **Sustain** — Existing coaching and maintained guidance

◆ At each decision gate: Proceed · Adjust · Pause. Care leadership makes each call, informed by Quality, Workforce, Learning, and Specialist Care. No fixed dates or durations.

ROLLOUT · PHASES 1–2

Prepare and Pilot

No Guide meets the intervention until required conditions are confirmed.

1 Prepare

Confirm everything Guides will depend on before any Guide meets the intervention.

MAJOR ACTIONS

- Validate the Guide Call Path against BH-4419 and approved guidance.
- Put approved Plus guidance, or an authoritative escalation route, in place, validated by Account Systems and Product Ops with Specialist Care.
- Confirm the Orbit eligibility workflow and General Care access.
- Validate the scenario; confirm Sana delivery, native or by approved link.
- Run the Lead working meeting for the pilot teams' Leads.
- Confirm the Guide → Lead support route and the established fallback.
- Select pilot teams, schedule practice time, and designate a participating Lead as temporary thread Admin.
- Quality sets the baseline comparison approach.

ACCOUNTABLE PARTNERS

Learning · Learning Operations · Care Systems · Account Systems & Product Ops · Specialist Care · Workforce · Care leadership · Quality

EVIDENCE / OUTPUTS

- Each readiness condition confirmed by its owner, or flagged missing.
- Comparison approach set; baseline findings on hand.
- Pilot Leads aligned on the sequence and coaching cues.

The optional Orbit serial-tool video is not a launch prerequisite.

◆ Are all required readiness conditions confirmed?

PROCEED	All five required conditions confirmed; optional items such as the Orbit video can follow. (1) approved Call Path and Plus guidance or escalation route; (2) validated Orbit workflow and access; (3) scenario delivery; (4) Lead alignment, a working support/fallback route, and a designated thread Admin; (5) Workforce-approved pilot group and practice time.
ADJUST	Readiness work is incomplete or needs correction. Finish or fix it, then re-check; launch waits.
PAUSE	A required condition can't be confirmed. Escalate to its owner; required items aren't worked around.

2 Pilot

Put the complete intervention in front of a representative group and confirm every part reaches Guides and works.

Proposed scope: 2–3 Lead teams with a representative mix of Guides, not volunteers or only high performers. Workforce confirms the Guide count, team selection, and scheduling.

MAJOR ACTIONS

- Leads run the team huddle with the slide, introduce the Call Path, and launch the scenario.
- Guides complete the scenario during scheduled work time.
- Guides use the Call Path on live WN7 calls right away, with their Lead as first-line support.
- Quality begins observing live WN7 calls as soon as they occur.
- Leads raise blockers in the shared Lead thread; the Admin routes them to owners.

ACCOUNTABLE PARTNERS

Team Leads · Workforce · Learning Operations · Quality · Care leadership

EVIDENCE / OUTPUTS

- Huddle attendance and scenario completion: delivery measures, not proficiency.
- Questions, blockers, and support requests logged.
- Early call observations, held for interpretation in phase 3.

◆ Is the intervention reaching Guides, and are its parts usable? (Execution is judged in phase 3.)

CONTINUE	Every component reaches Guides and works as designed. Observation continues.
ADJUST	A component isn't landing, such as scenario access or Lead availability. Fix it before judging execution.
PAUSE	Queue or Specialist capacity becomes unmanageable, or Guides lack a working support route.

Observe & Adjust, Expand, Sustain

Evaluate before expanding; expand wave by wave; sustain through existing coaching.

3 Observe & Adjust

Determine whether Guide execution is improving, and why or why not.

MAJOR ACTIONS

- Quality confirms the comparison method, and its limits, before interpreting pilot results, then leads review of pilot calls against the six indicators (page 4).
- Leads contribute coaching observations and diagnose before coaching: unclear next step, misread Orbit result, access or process blocker, or perceived pressure to keep troubleshooting.
- Coach the behavior or escalate tool and access problems; refine the Call Path, scenario, or cues where evidence shows a gap.

ACCOUNTABLE PARTNERS

Quality (lead) · Team Leads · Workforce · Learning · Care Systems

EVIDENCE / OUTPUTS

- Indicator-based comparison with the baseline.
- Breakdown causes and responses; refinement log.
- Recovery and operational readout; partner recommendations.

◆ With sufficient live-call exposure and comparable evidence, does execution support expansion?

PROCEED	Meaningful improvement in Guide execution, critical blockers resolved, capacity manageable, and customer recovery outcomes directionally promising.
ADJUST	The usual response to limited or mixed improvement. Refine and observe again.
PAUSE	Serious operational strain, inaccurate or unusable guidance, or unresolved critical access or support problems.

4 Expand

Extend in successive Lead-team waves, only as evidence and capacity allow.

MAJOR ACTIONS

- Expand by Lead-team wave, not as one organization-wide release.
- Align each Lead before their Guides enter; not all 18 Leads need to attend before the pilot.
- Repeat readiness checks per wave; carry refinements forward; keep observing with the same six indicators.

ACCOUNTABLE PARTNERS

Care leadership · Workforce · Learning · Quality · Team Leads

EVIDENCE / OUTPUTS

- Readiness, execution, and capacity readout for each wave.

◆ Every wave: does the evidence support it, and can operations absorb it?

PROCEED	Care leadership finds the evidence supports expansion **and** Workforce approves the wave.
ADJUST	A new wave shows different breakdowns. Slow down and address them.
PAUSE	Queue or Specialist capacity is strained.

5 Sustain

Keep execution reliable through existing coaching and maintained guidance.

MAJOR ACTIONS

- Leads reinforce the sequence through existing call reviews; Quality monitors recurring gaps.
- When BH-4419, Orbit, or Plus handling change, the operational owner validates and Learning updates the resources.
- Wind down temporary supports: the thread Admin role and Specialist thread participation.

ACCOUNTABLE PARTNERS

Team Leads · Care leadership · Quality · Learning · Operational owners

EVIDENCE / OUTPUTS

- Routine indicator-based observation; updates logged with owner and review date.

◆ Is execution holding through routine coaching?

MAINTAIN	Execution holds.
REVISIT	Procedures change or recurring breakdowns reappear.
RETIRE	Temporary supports, once routine coaching and escalation are enough.

MEASUREMENT & ACCOUNTABILITY

How we'll know it's working

Quality leads the evaluation, starting from evidence already in hand and comparing like with like.

Pre-rollout evidence base

204 eligible General Care calls, September 21–30. Source: systematic Orbit call sample; eligibility was classified retrospectively.



76 complete recoveries (37%) **13** incomplete recoveries (6%) **115** generic troubleshooting (56%)

Established pre-rollout findings, not scores against the six indicators. Before launch, Quality sets a defensible comparison method. Where underlying call evidence is available, Quality reviews baseline calls against the indicators; where it isn't, Quality states the baseline's limits and doesn't claim indicator-level improvement it can't establish.

Six observable indicators of successful execution

What Leads and Quality look for in reviewed WN7 calls, applied where relevant.

Recognition
Recognizes the potential WN7 pattern and moves toward verification

Verification
Checks eligibility and correctly interprets the result

Decision
Selects the appropriate path based on verified eligibility

Recovery
Completes both applicable transactions, replacement and Plus, or establishes documented follow-up

Communication
Accurately explains what was completed, what's pending, and what happens next

Documentation
Records completed actions, outstanding work, and follow-up ownership

> **"Once the path is live, the important questions will be: did they recognize the pattern, check eligibility, complete both transactions, and explain what happens next?"**
> Malik, Quality reviewer, September 15 calibration comment

Four kinds of evidence, read together

Delivery
Huddle attendance; scenario access and completion.
Shows reach. Not proficiency.

Guide execution
The six indicators, from reviewed calls.
Shows what Guides did on the call.

Customer recovery
Complete and incomplete recovery; repeat contacts, where measurable.
Shows whether the recovery was documented as complete; repeat contacts may indicate whether the resolution held.

Operational signals
Handle time; wait and abandonment; Lead and Specialist load.
Shows capacity, not whether a customer got the right resolution.

Recovery and operational measures are proposed, pending Quality and Workforce confirmation.

What the Evidence Can (and Can't) Tell Us

- ✓ Same indicators, comparable call selection, applicable eligibility branches, and explicit treatment of missing evidence.
- ✓ Observation starts with the first live calls; conclusions wait for sufficient exposure and comparable evidence.
- ✗ Attendance or scenario completion as proof of proficiency.
- ✗ A recorded Orbit check as proof the Guide understood the result.
- ✗ Changes in operational metrics as proof the intervention caused them.
- ✗ Numeric pass marks; progression rests on accountable judgment.

Progression authority: two approvals before any wave proceeds

Care leadership
Decides the evidence supports expansion, informed by Quality, Learning, and Specialist Care.

+

Workforce
Approves the wave's composition, timing, scheduled practice coverage, and operational feasibility.

=

Next wave proceeds
Each Lead aligned before their Guides start; refinements carried forward.

OWNERSHIP & DEPENDENCIES

Responsibilities Across the Rollout

Proposed responsibilities, pending confirmation. Beacon remains authoritative for procedures.

AREA	OWNS	CONTRIBUTES	VALIDATES / APPROVES
Rollout progression	Care leadership (proceed, adjust, pause)	Quality, Workforce, Learning, Specialist Care	Care leadership; each wave also needs Workforce approval
Pilot and wave composition	Workforce: composition, timing, practice coverage	Coverage and capacity data	Workforce approves each wave's feasibility
Evaluation	Quality	Leads: coaching observations, diagnosis · Workforce: capacity data	Recommendation to Care leadership
Lead working meeting	Learning facilitates	Care leadership ensures participating Leads can attend	Specialist support only with Mateo's approval (A)
Guide resources: Call Path, scenario, huddle slide	Learning	Quality findings; practice and coaching feedback	Operational owners for procedures, Plus, and Orbit content
Lead resources: one-pager, Coaching Cues	Learning	Quality findings; Lead feedback	Quality and Specialist Care validate operational content
Scenario delivery (Sana)	Learning Operations	Learning: scenario content	Learning Operations confirms
Live Guide support	Care leadership: the arrangement	Leads: first-line support · established fallback route remains	Workforce validates coverage; Specialist Care confirms escalation, subject to capacity
Recovery procedures	Specialist Care / policy owners	Complex recovery issues from the field	Validated within their authority; published in Beacon
Plus procedures and transaction-status guidance	Account Systems and Product Ops validate	Specialist Care: operational expertise	Final policy authority to be confirmed
Orbit access and tool behavior	Care Systems	Access and tool blockers raised by Leads	Care Systems validates
Shared Lead thread (temporary)	A participating Lead as Admin	Lead questions and blockers	Care leadership designates the Admin
Reinforcement and monitoring	Team Leads, Care leadership	Quality monitors recurring gaps	—

Two separate Specialist approvals

Mateo (Specialist Care) decides each separately; neither implies the other.

A · Lead working meeting
Support for small-group case discussion.

B · Temporary thread support
Clarify from approved guidance, sized to capacity.

If capacity allows only one, the Lead meeting comes first.

Temporary shared Lead thread

During the initial rollout, Leads need a shared place to surface questions, resolve conflicting guidance, and identify recurring blockers. A temporary coordination thread keeps those discussions visible and routes them to the appropriate owners, rather than creating another informal source of answers.

The Admin coordinates questions, routes blockers to owners, tracks recurring patterns, and circulates verified answers.

For Lead coordination only. It doesn't replace live Guide → Lead support or established escalation, and it isn't a permanent channel. It winds down in Sustain.

Confirmations and when they're needed

BEFORE PILOT LAUNCH · REQUIRED

- **Call Path and scenario content** · Learning, validated by operational owners
- **Plus guidance or escalation route** · Account Systems, Product Ops, Specialist Care
- **Orbit workflow and General Care access** · Care Systems
- **Scenario delivery (Sana or approved link)** · Learning Operations
- **Lead alignment, live support route, thread Admin** · Care leadership, with Workforce
- **Pilot teams and practice time** · Workforce

NOT LAUNCH BLOCKERS · CONFIRM WHEN NOTED

- **Recovery and operational measures** · Quality and Workforce, before evaluation
- **Baseline comparison method and its limits** · Quality, before results are interpreted
- **Specialist approval A** · Mateo, before the Lead meeting, if Specialists attend
- **Specialist approval B** · Mateo, before Specialists join the thread